



Stoma Management – Faecal and Urinary Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	All clinical services	Nursing, Midwifery and Medical

2. Purpose

The purpose of this document is to outline the requirements for Stoma Management at Armada Kalamunda Group (AKG).

Relevant faecal and urinary stoma management standards should be applied in accordance with the needs of the individual patient.

Stoma management should be established on admission of the patient or following surgery when a stoma has been formed.

Where specific care or requirements vary, staff should seek review from the Clinical Nurse Specialist Stomal Wound Care (CNS) or senior clinician advice, if the CNS is unavailable.

3. General information

A stoma is a surgically formed diversion using bowel brought through the abdominal wall diverting either stool or urine. The word stoma derives from the Greek word for opening or mouth. Examples of stomas include colostomy, ileostomy, and urostomy/ileal conduit.

A colostomy is a bowel diversion of the colon. It may be formed from any section of the large bowel. Its position along the colon will dictate the output and consistency of faeces.

An ileostomy is a bowel diversion formed when a section of the ileum is brought out onto the abdominal wall. It can be formed anywhere along the small bowel. The output will be a liquid stool (ideally custard to toothpaste consistency).

An ileal conduit (urostomy) is a short segment of ileum to be resected to act as a conduit for urine. The ureters are implanted into the segment of ileum and the ileum is then matured through the abdominal wall. Bowel continuity is maintained with a primary anastomosis within the ileum.

4. Pre-procedure management

The CNS should be informed of all patients for potential stoma formation prior to surgery. The CNS will:

- Counsel patient and significant others as appropriate
- Provide education and support
- Undertake pre-operative stoma siting

- Liaise with surgical team as indicated
- Arrange for funding for pouches through the Stoma Appliance Scheme (if eligible)

5. General stoma observations

Perform stoma observations a minimum of each shift and with increased frequency if clinically indicated. Assessment outcomes at each appliance change should be clearly documented.

5.1 Colour

- A stoma should appear pink/red, moist, and shiny in appearance indicating good blood supply.
- Report pale, dark, dusky, or black stomas to the Shift Coordinator/STN/MO immediately.

5.2 Oedema

- Observe and report oedema in the presence of colour changes e.g., pale, dark, dusky.
- During the initial post-operative period the stoma may appear oedematous, but it should decrease in size over 6 – 8 weeks.

5.3 Bleeding

- Surface mucosal bleeding will occur in view of the vasculature of the bowel. Topical application of a hydrocolloid powder will act as a haemostat.
- Active bleeding with reference to stoma output should be reported immediately.

5.4 Protrusion

An ideal stoma with liquid output (ileostomy/ileal conduit) should protrude 2 – 3 cm from skin level to facilitate the effluent draining into the pouch. A colostomy should protrude slightly but may also be flush with the skin as the output is of a thicker, more formed consistency

Stoma protrusion is described as:

- Retracted (below skin level) - report any signs of retraction
- Flat/flush (level with skin surface)
- Moderately protruding (1-3cm)
- Long/well spouted (>3cm)
- Prolapsed (measure level of extension).

5.5 Mucocutaneous junction

The mucocutaneous junction (MCJ) is the border between the stoma and the abdominal skin.

- It should be intact with evenly placed intermittent sutures.
- Sutures are usually dissolvable and will fall out over time.
- Observe and report any mucocutaneous separation.

5.6 Peristomal skin

The peristomal skin is the skin immediately surrounding the stoma and should appear similar to the skin on the other side of the abdomen, it should be intact.

Impaired skin integrity may indicate appliance leakage or other complications, requiring review.

When the appliance has been removed, there may be some transient erythema. However, it should not remain red or be painful.

Observe for redness, erythema, rashes, impaired skin integrity and pain/tenderness. Report any abnormalities to Shift Coordinator/CNS/medical team.

5.7 Effluent Produced

Observe amount, colour, and consistency from the stoma.

Record on the Fluid Balance Chart and in the patient integrated notes as appropriate. The output from any newly formed stoma must be recorded.

6. Specific Stoma Observations

6.1 Ileostomy

- Assess for passage of flatus which indicates that bowel function is resuming.
- Output commences as haemoserous or bile stained.
- Stool output can be up to 2000mL in 24hours but should reduce to 500mL-1000mL per 24hours after the initial post-operative period.
- A patient with an ileostomy can rapidly become dehydrated and develop major electrolyte imbalances if output is excessive. An accurate fluid balance chart is essential and should capture volume, colour, and consistency of the output.
- Output should be assessed for medication malabsorption in the presence of high output, whereby medications may be visible.
- If an ileostomy is inactive for 4-6 hours and is accompanied by abdominal discomfort/pain, cramps, distension, nausea and/or vomiting, contact the medical team.

6.2 Ileostomy obstruction

- Defined as a blockage of the ileostomy, indicated by partial or complete cessation of output.
- Can be caused by high-residue foods such as fibrous green leafy vegetables, pineapple, nuts, coconut, corn and by internal changes e.g., adhesions.

Suggested actions include:

- Observe for swelling of the stoma
- Encourage a warm bath or shower to relax abdominal muscles
- Change body positioning e.g., knee to chest may encourage movement of bolus of food
- DO NOT give laxatives. Medical advice should be sought.

Paralytic Ileus/Small Bowel Obstruction

Symptoms are more severe and oral fluids generally cannot be tolerated; may be an absence of flatus. Seek urgent medical attention

6.3 Colostomy

- Assess for passage of flatus, indicating that bowel function is resuming.
- Output commences as haemoserous fluid then changes to liquid stool, to semi-formed and then to a formed consistency. As diet intake increases the output will become thicker and more formed. Volume of food and types of food consumed will influence consistency and frequency of output.
- A patient with a colostomy can be prone to constipation and bowel actions should be recorded. If the patient becomes constipated, they may need review for aperients.

- The stoma may not always be active of faecal output every day, but there should be signs of flatus in patients with a colostomy.
- If a colostomy is inactive for more than 24 hours and is accompanied by abdominal discomfort/pain, distention, nausea and/or vomiting contact CNS/medical team.

6.4 Urostomy or Ileal Conduit

- Urine should be present commencing immediately after urostomy formation.
- The absence of urine from a urostomy/ileal conduit is deemed as an emergency and requires urgent review by the treating team
- Urine may be blood stained immediately post-surgery but changes to a clear straw colour. Variable amounts of mucous will be present due to the segment of bowel used to create the conduit.
- An accurate fluid balance chart is essential – colour, volume and consistency of urinary output should be recorded.

6.5 Bridges/Stents

- Sometimes a plastic or latex bridge is used to support a newly formed loop stoma. This can help to prevent retraction of the stoma.
- The bridge is usually removed by the CNS or competent staff after 5 days, or as per Surgeon's preference
- Stents are used in a newly formed Ileal Conduit to minimise the risk of stenosis at the junction of the ureters with the segment of ileum forming the conduit. Stents remain in-situ for a few weeks, they can be removed as per Surgeon's preference, or they sometimes fall out. Document if bridges or stents are in-situ.



Stoma with bridge



Ileal conduit with stents

7. Patient monitoring

- Requirements of a stoma assessment depends upon whether the stoma is newly formed or has been there long term.
- Any identified abnormalities or problems should be referred to the CNS, who will develop a customised management plan.
- Assessment can be carried out by medical, nursing, and allied health staff.
- All interventions should be documented and evaluated to ensure issues have been resolved.
- Individualised management plans for new Ostomates should be documented in the patient's records to facilitate education and continuity of care.
- Established ostomates should ideally be reviewed annually in an outpatient setting or earlier if complications arise.

8. Patient education

Initial teaching is usually commenced by the CNS and is reinforced by nursing staff on the wards. Time and patience are necessary for the patient to assimilate new skills and adjust to the change in their excretory function. Patients should be encouraged to participate in their own stoma care when able.

Patient education includes:

- Explaining the procedure
- Equipment preparation
- Patient preparation - the bathroom is the ideal place as it provides washing facilities and privacy
- Removal of appliance
- Skin care
- New stomal appliance: preparation, application, and disposal.
- Dietary advice. Referral to a Dietitian is recommended for Ileostomies for education on diet/hydration strategies to minimise the potential for complications such as high output with electrolyte imbalance/dehydration and food blockages.

9. Stoma appliance change

First appliance change is performed 48-72 hours post-operatively or if appliance is leaking. Liaise with the CNS for advice.

9.1 Equipment

- Clear drainable 'cut-to-fit' post-operative pouch, without a filter to allow for observation of the stoma and output. As there is no filtration system, the appliance will balloon with the presence of flatus, indicating gut motility.
- Once the patient has passed flatus select the stoma appliance best suited to clinical requirements, products available and patient preference.
- Disposable gloves (non-sterile)
- Warm water
- Rubbish bag
- Absorbent Sheet ('Bluey')
- Soft wipes (tissues or soft paper towels)
- Receptacle (kidney dish/jug)
- Curved scissors
- Stoma measuring guide
- New appliance
- Adhesive remover sachet/ spray

9.2 Procedure

Stoma care is NOT a sterile procedure

- Explain procedure and obtain patient consent. Promote a positive attitude.

- Ensuring privacy, assist the patient into a comfortable position in the bed or chair (as clinically appropriate). Standing, in front of the bathroom mirror if general condition permits, will facilitate access and visibility for accurate alignment of the appliance over the stoma.
- Wearing gloves empty the stoma pouch into a kidney dish or jug and record volume, colour & consistency on the fluid balance chart (if the patient is wearing a drainable pouch).
- Remove the pouch, starting from the top edge and peel off in a downward direction using an adhesive remover wipe or spray.
- Gently press the peristomal skin away from the pouch while removing the appliance. This will minimise the shearing effect of removing the adhesive from the skin.
- Place the used stoma pouch in a rubbish bag and discard in the general refuse bin at the end of the change. (Stoma pouches are made of plastic and are not biodegradable and should not be flushed down a toilet).
- If gloves are soiled, change them, and perform hand hygiene and don clean gloves.
- Cleanse the stoma and the surrounding peristomal skin using warm tap water and a disposable soft cloth or tissues. Do not use pre-moistened wipes. Pre-moistened wipes, perfumed or baby wipes contain cleansers, moisturisers and alcohol which can prevent the pouching system from adhering
- In the presence of stomal oedema, cleansing with cool water will cause the stoma to retract and will facilitate application of the new pouch.
- Surface mucosal bleeding may occur secondary to the vasculature of the bowel, irrespective of a gentle cleansing technique. A dusting of hydrocolloid powder will arrest this response; the powder should be brushed off intact skin if it disperses onto the skin, as it will disrupt optimal adhesion of the new appliance.
- Dry carefully around the stoma and peristomal skin with paper towel or disposable soft cloth. The peristomal skin needs to be dry to enhance adhesion of the new appliance.
- Measure stoma size using a measuring guide allowing a 1mm- 2mm gap of skin to be seen around the stoma (If the aperture/hole is too big stoma output will erode the peristomal skin).
- Using curved scissors, cut out the base plate to the correct size. Creating a template for irregular shaped stomas will facilitate an accurate fit.
- Smooth the cut edge of the base plate with your finger.
- Remove adhesive backing and apply the pouch starting at the base of the stoma and then fit over the stoma.
- Ensure the pouch adheres smoothly with no creases and conforms to the peristomal skin to prevent leakage.
- If using a drainable pouch, ensure that the outlet is closed.
- If using a two-piece system, secure pouch to baseplate.
- To promote adhesion, encourage the patient to place hand over the appliance for 5 to 10 minutes and avoid bending in the middle or have a shower or bath for 10 minutes post application.
- Dispose of used equipment and PPE appropriately.
- Ensure the patient is comfortable.

10. Documentation

- Failure to record accurately and legibly in patient health records can contribute to a decrease in the quality and safety of patient care.
- All stoma observations and stoma appliance changes are to be documented/updated as appropriate in the Patient integrated notes, Nursing care plan and Fluid balance chart

11. Discharge Planning

All patients with a newly formed stoma are to be reviewed by the CNS during hospitalisation and prior to discharge. The CNS will provide:

- Education, training, and counselling pertaining to individual management to patient and significant others
- Access to the Stoma Appliance Scheme (if eligible) through the Western Australian Ostomy Association to enable access to ongoing stomal supplies
- Refer to Silver Chain for stoma support if eligible
- Specific educational resources
- Appropriate outpatient follow-up appointments with a Stomal Therapy Nurse

12. Legislative context

The following documents are required to give effect to this procedure:

- HDWA [Clinical Handover Policy MP 0095](#)
- East Metropolitan Health Service (EMHS) [Patient Identification and Procedure Matching EMHS: 21](#)

13. Supporting information

The following documents support the implementation of this procedure:

- [Advanced Health Director and Enduring Power of Guardianship AKG-GUI-0013-18](#)
- [Wound Management Procedure AKG-PRO-0476-15](#)
- [Recognition and Response to Acute Deterioration Policy AKG-POL-0388-14](#)

14. Compliance, monitoring and evaluation

Compliance with this policy will be monitored via:

- Monitoring of reported clinical incident data via Datix CIMS.

Department specific findings are to be reported at the relevant departmental meeting for review and identification of any opportunities for improvement.

15. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.17 Comprehensive care at end of life
- Standard 5 – Action 5.28 Nutrition and hydration
- Standard 6 – Action 6.5 Correct identification and procedure matching
- Standard 6 – Action 6.7 Clinical handover

- Standard 8 – Action 8.4 Recognising acute deterioration

16. Glossary

The following definitions are relevant to this policy.

Term	Definition
Stoma	Stoma is a Greek word meaning 'mouth' and is used to describe an artificially created opening from the gastrointestinal or urological tracts.
Ostomate	A person who has undergone a surgical procedure to create an opening in the body that will be used to excrete urine or faeces.
CNS	Clinical Nurse Specialist Stomal Wound Care
Colostomy	An opening into the Colon (Large Bowel)
Ileostomy	An opening into the Ileum (Small Bowel)
Urostomy	An opening into the Urinary tract, most commonly as an Ileal conduit

17. Policy owner

Enquiries relating to this procedure may be directed to:

Title: Denver Prince - Coordinator of Nursing Surgical Services
 Email: Denver.prince@health.wa.gov.au

18. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4.1	31/07/2025	18/03/2027	Minor amendment – appendix 1 added

19. Authorisation

This procedure has been authorised and issued by the Director Nursing & Midwifery

Authorised by	Jane Husain – A/Director Nursing and Midwifery
Authorisation date	31/07/2025
Published date	31/07/2025

Appendix 1: Stoma observation prompt sheet

Colour of the stoma: Necrotic, Dusky, Red, Pink, Pale/Anaemic

Oedema: Is the stoma very/mild/minimal oedematous, or not oedematous.

Bleeding: Yes/No. Location of bleeding. Eg at the mucocutaneous junction, bleeding mixed with faecal matter.

Leaks: - Yes/No. When + where did the leak occur.

Active: Yes/No. Mark consistency on Bristol stool chart, not writing stoma active

Output in 24hrs: <200ml, 300-600ml, 700-1200ml, >1300ml.

- Consistency, any undigested tablets visible.

Stoma Profile: Moderately spouted (1-3cm), Well spouted (<3cm), Flat/Flush, Retracted (below skin level), Prolapsed (measure level of extension)

Measurement: If stoma being changed, what size is it. Use the measuring tool. Size in mm

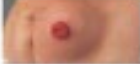
Mucocutaneous Junction: Intact, dehisced, if newly formed, dissolvable sutures will be visible.

Location of Dehiscence (if applicable): Describe the location of the dehisced area that correlates with clock times.

Peristomal Skin (If changing the appliance): Intact/Healthy, Mild erythema, Erosion, Allergic reaction, Ulceration, Fungal infections, Folds/Creases, granulomas. Any c/o pain.

Other: Is patient aiding in stoma management. Did anything need to be escalated to the treating team.

Ostomy Forum - Observation index

Stoma	Status	Skin	Condition
	Normal	▶	above skin level
	Flush	▶	mucosa level with the skin
	Retracted	▶	below skin level
	Prolapsed	▶	notable increasing length of stoma
	Hernia	▶	bowel entering parastomal space
	Stenosis	▶	tightening of stoma orifice
	Granulomas	▶	nodules/granulation on stoma
	Separation	▶	mucocutaneous separation
	Recessed	▶	stoma in a skin fold or a crease
	Necrosis	▶	lack of blood supply causing partial or complete tissue death
	Laceration	▶	mucosa that is jagged/torn or ulcerated due to trauma
	Oedematous	▶	gross swelling of the stoma
	Others	▶	
	Normal	▶	as rest of skin
	Erythema	▶	red
	Macerated	▶	excoriated; moist
	Eroded	▶	excoriated; moist and bleeding
	Ulcerated	▶	skin defect reaching in to subcutaneous layer
	Irritated	▶	irritant causing skin to be inflamed, sore, itchy and red
	Granuloma	▶	nodules/over granulation tissue on skin
	Predisposing factors	▶	underlying diseases, e.g. eczema and psoriasis
	CPD*	▶	greyish, nodules on skin, often caused by urine
	Infected	▶	e.g. fungus, folliculitis
	Others	▶	e.g. Pyoderma gangrenosum

* Chronic Papillomatous Dermatitis